

D11 – Office Visit Note

P.A. Kim, MD | Family Medicine | 22 Jan 2015

Patient: Caduceus, Opus J. MRN: 004-7703 DOB: 14 Mar 1956 Allergies: NKDA

OFFICE VISIT NOTE

Patient: Opus J. Caduceus | MRN: 004-7703

Date: 01/22/2015 | Provider: P.A. Kim, MD

Visit Type: Established Patient / Follow-Up

Practice: Oak Harbor Family Medicine (TRICARE), Oak Harbor WA

CHIEF COMPLAINT: Pre-operative medical clearance for repeat Nissen fundoplication. TRUS biops

HISTORY OF PRESENT ILLNESS:

58-year-old active duty USN with refractory GERD despite high-dose PPI and prior Nissen fundoplication (Sep 2009). Repeat EGD ordered in fall 2014 by Dr. Abara demonstrated recurrent hiatal hernia and LA grade B esophagitis without strictures identified. Repeat Nissen fundoplication scheduled for 12 Feb 2015 at Swedish Medical Center with Dr. Abara. Here today for pre-operative medical clearance and to review the TRUS prostate biopsy result obtained in September 2014. No new cardiopulmonary symptoms. Functional capacity preserved (>4 METs; patient reports continuing to climb stairs without difficulty and performs occupational duties without limitation). No bleeding history.

CURRENT MEDICATIONS:

- Omeprazole 40 mg PO daily
- Metoprolol succinate 25 mg PO daily
- Rivaroxaban 20 mg PO daily
- Tamsulosin 0.4 mg PO daily
- Topiramate 25 mg PO daily
- Naproxen 500 mg PO BID PRN
- Flunisolide nasal spray PRN
- CPAP 10 cmH2O nightly

PHYSICAL EXAMINATION:

Vitals: HR 68 | BP 130/78 | RR 14 | SpO2 97% RA | Wt 88 kg

General: NAD. Well-appearing.

HEENT: Mallampati III. No oropharyngeal lesions.

CV: Rate-controlled, regular. No murmurs, rubs, or gallops. No JVD.

Lungs: Clear bilaterally with good air movement.

Abdomen: Prior surgical scars well-healed. Soft, non-tender. No HSM.

Ext: No edema. Distal pulses 2+ symmetric.

ASSESSMENT & PLAN:

1. GERD / hiatal hernia, refractory - despite PPI and prior Nissen (Sep 2009). Repeat EGD (Nov 2014, Dr. Abara, Seattle): recurrent hiatal hernia, LA grade B esophagitis, NO STRICTURES IDENTIFIED. Repeat Nissen fundoplication scheduled 12 Feb 2015, Swedish Medical Center. MEDICALLY CLEARED today for surgery from PCP perspective. Cardiology cleared by Dr. Mensah on 08 Jan 2015. Hold rivaroxaban per surgical anticoagulation protocol (per Dr. Abara). Resume post-op when surgical team clears. Clearance letter sent to Dr. Abara.
2. Prostate biopsy result review - 12-core TRUS biopsy (17 Sep 2014, Dr. Tran, Olympic Urology Bremerton): benign prostatic tissue, no malignancy identified. Gleason N/A. BPH confirmed. Continue tamsulosin 0.4 mg daily. PSA surveillance every 12 months.

3. Migraine headaches - improved on topiramate 25 mg daily. Average frequency now approximately every 6 to 8 weeks with shorter duration. Neurology stable. Continue current regimen.
4. Atrial fibrillation, OSA - all stable on current regimens. Cardiology and sleep medicine follow-up current.

Follow-up: Post-op follow-up 4-6 weeks after surgery. PCP RTC 6 months.

Electronically signed by: P.A. Kim, MD
Date: 01/22/2015 10:15

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